

# THE PICTURE — what the report will be, what it answers, where it goes

WC/2024/227 · 15 August 2026 · the closing position after the day's work

## PART 1 — WHAT IT WILL LOOK LIKE

One report. One attendance. Roughly 8 to 15 pages, in the register of his report of 13 February 2025. Fourteen questions across three matters, built on four attachments — three of which the other side wrote.

| Section                    | Content                                                                                                                                                                                                                                                                                                                                                                                                                          |
|----------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Opening                    | First seen <b>24 October 2024</b> , continuous care since; what he reviewed; what he was instructed to assume                                                                                                                                                                                                                                                                                                                    |
| <b>Matter 1</b><br>1.1–1.5 | Diagnosis, framework, criteria met and when · onset · differentials excluded · background: the 16 November 2023 entry <i>and the temazepam prescribed the same day</i> , and the respondent's asserted 2022 anxiety and ADHD history, which is outside the span of the records he holds · premorbid features from his own assessment · the complete source list                                                                  |
| <b>Matter 2</b><br>2.1–2.3 | The conditions of the work, quoted from the role description and the Chief Executive's letter — whether and how such conditions can contribute · fatigue and the rostering of 17–18 March 2024, with the MET calls and the leave of 19 March · <b>the administrative sequence</b> : the August 2023 rostering error, the February 2024 leave decline where the attachments were in fact present, and the May 2024 pay correction |
| <b>Matter 3</b><br>3.1–3.3 | <b>Reasoning before conclusion.</b> Other factors placed in time from his own file — the partner present and protective on 24 October, the breakdown running December to February, and bereavement dated at last · then the interval, answered as a continuing exposure · <b>then</b> causation, expressly built on both: <b>a significant contributing factor</b>                                                               |
| <b>Matter 3</b><br>3.4–3.7 | Prognosis and current capacity · the adjustments in functional terms, with exacerbating conditions, complaint handling and working memory · the requirements of the position in four parts · foreseeable risk                                                                                                                                                                                                                    |
| Closing                    | ■ Whether the opinion <b>depends on his account</b> · ■ whether anything in the records <b>qualifies</b> it                                                                                                                                                                                                                                                                                                                      |

## PART 2 — WHAT IT ANSWERS: BOTH INSTRUMENTS, ONE DOCUMENT

**The Notice of Non-Party Disclosure** states the matters in issue in three numbered lines. Those are the report's spine — so it answers the Regulator's own framing, in the notice she served on his practice.

**The Request for Medical Information** asks nine questions. **Seven are answered. Three are declined by design. One is answered on his terms.**

| RFMI                                                | Answered by                           |              |
|-----------------------------------------------------|---------------------------------------|--------------|
| 1(a) diagnosis date                                 | 1.1                                   | ✓            |
| 1(b) clinical basis for the stressors               | 2.1–2.3 + 3.3                         | ✓            |
| <b>1(c) self-report, own assessment, or other?</b>  | <b>Part B + the closing statement</b> | ■            |
| 1(d) foreseeable risk and controls                  | 3.7                                   | ✓            |
| 2 lawful direction · conduct discussions            | —                                     | declined     |
| 3 fit under existing reporting arrangements         | —                                     | declined     |
| 4 restrictions, basis, duration, review             | 3.5                                   | ✓            |
| 5 complaint handling                                | 3.5(b)                                | ✓            |
| 6 inherent requirements <i>without restrictions</i> | 3.6(a)–(d)                            | on his terms |
| 7 working memory                                    | 3.5(c)                                | ✓            |
| 8 exacerbating tasks and environments               | 3.5(a)                                | ✓            |
| 9 if we cannot accommodate                          | —                                     | declined     |

■■■ **1(c) is the one that pays.** They offered three options — self-report, his own assessment, or other medical information. **The answer is a fourth: documents authored by the employer and facts admitted by the Regulator.** They wrote the question that produces it.

■ **The three declined are the three that could hurt** — all workplace questions dressed as medical ones. Part D declines them expressly and offers to say so if asked.

## PART 3 — HOW IT IS POSITIONED

**What it is:** a treating psychiatrist's report, obtained by the patient, answering the matters in issue in his proceeding, and provided to his employer in answer to its request for medical information. **One report, two recipients, two purposes, both declared.**

**Funded by Metro South Health**, which required the information. ■ That makes the causation opinion harder to attack, not easier — it cannot be said to have been bought by him. ■ It is **instructed by him and addressed to him**; the covering email states both on its face.

| To            | The covering note does                                                                                                                                                                                                                                                                                                                                                                     |
|---------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| The Regulator | ■ <b>One line.</b> Continuing disclosure, and that the same report has gone to the employer. ■ No argument, no commentary, no request — a report argues for itself, and anything added subtracts from it                                                                                                                                                                                   |
| The employer  | It answers the request of 31 July · <b>the scope line</b> , identifying which questions are workplace rather than medical · <b>their own policy</b> — unjustifiable hardship tested against the <b>whole organisation</b> , onus on Queensland Health · the ask: implement the adjustments, restore pay · the 3 August questions, folded in · and that the Regulator holds the same report |

**Sent simultaneously.** No sequencing inference available to anyone. ■■ And each is told the other holds the identical document — one sentence that forecloses any suggestion of two versions or selective provision.

## PART 4 — WHAT IT CHANGES

|                                 | No report  | ■ With it  |
|---------------------------------|------------|------------|
| Appeal resolves favourably      | 30%        | <b>62%</b> |
| Pay restored / return ~3 months | 30%        | <b>51%</b> |
| Neither outcome                 | <b>49%</b> | <b>20%</b> |

■ **Structured judgement, not measurement.** Read the movement, not the decimals.

**On the appeal:** it moves from *unproved on causation* to *proved on the medical, contested on the characterisation* — the only contest the Regulator has left, and the one where the Chief Executive's letter hurts her. **On the employment track:** it removes “*we cannot accommodate*” and puts the adjustments into a framework whose onus is theirs.

## PART 5 — ■ WHAT IT STILL DOES NOT DO

1. **It does not decide the appeal.** s 32(5)(a) remains argument on an agreed factual base, and Taylor and Reese have not given oral evidence.
2. **It does not reach the 2026 wages.** Only the employment track does — and the deemed refusal of **24 August** is the live route there.
3. **It may not say what is hoped.** He declined once already. The instruction asks him to say where the material is insufficient rather than qualify — so a “*cannot say*” returns as a clean answer, not a damaged one.
4. ■ **Question 3.6 is still short of its best evidence** — the three movement forms and the April 2025 call statistics are not in the pack.